

Advisor Questions — Jessica Solomon

Who she is: ~20 years in senior-care business development across home health, hospice, and caregiver referral networks.

Why she's the right person to pressure-test: Kinroster's entire early-growth bet is *relationship-driven, not ad-driven* — state associations, Facebook operator communities, placement consultants, peer referrals, and a possible Synkwise partnership, all founder-led (docs/business/go-to-market.md). Jessica has spent two decades building exactly these referral-driven distribution motions in senior care, so she can stress-test our riskiest assumptions better than almost anyone.

How to use this doc

- Lead with the open-ended questions in Sections 1–4 and let her **disagree** — the goal is to find where our plan is wrong, not to get validation.
- Keep the "would you advise us / open doors" questions (Section 8) for the back half, once there's rapport and she's already invested in the problem.
- You don't need all of these. Pick 6–8 that matter most for this specific call.

How to introduce the idea to her

(She's an industry insider, not a customer — don't pitch, frame a problem she knows cold and invite her read. Lead with the outcome, not "AI." Be honest about stage.)

A 30-second opener you can say out loud:

"I'll keep this short because I really want your take, not to pitch you. Small residential care homes — the 6-to-20-bed RCFEs and adult family homes — are still running documentation on paper binders and group texts. Owners burn a couple of hours a day rewriting caregiver notes, and families are left in the dark until something goes wrong. We're building a tool that turns a caregiver's quick spoken or typed note into a clean, structured shift record, flags incidents, and keeps families informed — so the home documents better, passes its survey, and the owner gets their evenings back. We're early — pre-launch, lining up our first pilot facilities. You've spent 20 years getting small operators to actually adopt things, so I mostly want to know where this is wrong and how it really gets into their hands."

Principles behind that opener:

- **Sell the outcome, not the tech.** "Documents better, passes survey, gets evenings back" — not "AI documentation platform." (Per docs/marketing/launch-strategy-2026-05.md, the buyer doesn't care that it's AI and may be put off by it.)

- **Name the buyer precisely** so she immediately pictures the operators she knows.
- **Be honest it's early.** With an advisor, "pre-launch, seeking pilots" earns more candor than overstating traction.
- **Make the ask "tell me where I'm wrong,"** not "will you buy / partner." That comes later (Section 8) once she's engaged.
- **Keep it under a minute** so the rest of the call is her talking, not you.

One-liner version (if she just asks "what is it?"):

"It's documentation and family-communication software built specifically for small care homes — it turns a caregiver's quick note into a clean shift record and a family update, so the home stays survey-ready without the owner doing it all by hand."

1. Warm-up / her lens

(Gets her talking, and quietly reveals how she'd think as an advisor.)

- In 20 years across home health, hospice, and referral networks, what actually moved the needle on getting a small operator to adopt something new?
- Where do you see small residential care homes (6–20 beds) heading over the next 3–5 years — consolidating into portfolios, professionalizing, or getting squeezed out?
- What do most tech founders get wrong when they try to sell into senior care?

2. ICP / buyer reality

(Tests market-analysis.md: owner-operator of a 6–16 bed RCFE, \$100–300/mo budget, findable in Facebook groups.)

- Is the 6–16 bed RCFE genuinely owner-operated, or increasingly run by hired admins and small multi-home portfolios? Who really signs off on a \$149/month tool?
- Does that owner buy because it "saves me 1.5 hours a day," or because of cash-flow pressure or fear of a citation? What's the actual emotional trigger to purchase?
- When this buyer wants a recommendation they trust, where do they actually go?

3. Distribution channels — the core pressure-test

(Tests the ranked channel strategy in go-to-market.md.)

- **Associations (CALA, Adult Family Home councils, etc.):** Do operators actually act on vendor sponsors and presentations at these, or is it mostly noise? What's a realistic close rate from an association director's warm intro?
- **Facebook operator groups:** Do RCFE owners really refer purchases to each other

peer-to-peer, or is that overstated? How fast does word-of-mouth actually move in this market?

- ****Placement consultants / geriatric care managers (A Place for Mom, ALCA) — your home turf:**** Would they ever recommend an operator's back-office tool? What's in it for them? How do those referral relationships really get built and kept alive?
- If you had to kill one of our channels and double down on another, which and why?

4. The referral-network playbook

(Mine her specialty directly — this is where her experience is most unique.)

- How did you personally build a referral engine in home health or hospice from zero? What were the first moves?
- What does a referral partner need to see before they'll put their own name on you?
- How do you keep a referral relationship alive without paying for every single lead?
- We're planning a 20% first-year referral commission (capped at 24 months). Is that enough to actually drive referral behavior, or do happy operators just stay quiet?

(tests the incentive structure in go-to-market.md)

5. Pricing strategy

(We have NOT settled on a price — market-analysis.md floats \$149/mo flat, but the goal of this section is to use her to figure out the right number and model, not to validate ours. Hold the \$149 figure back at first; ask her to price it cold, then see where she lands versus our hypothesis.)

- If a tool saved a small RCFE owner a couple of hours a day on documentation and kept families informed, what would you expect them to actually pay per month? Where's the ceiling before they balk, and where's the floor where they assume it's junk?
- How does this buyer reason about a recurring software bill — per facility, per bed, per resident, per caregiver seat? Which model do operators understand and resent least?
- Is flat monthly pricing right for a home with variable beds/staff, or do operators expect to "pay for what they use"? What have you seen work in home health / hospice?
- What's the real anchor in their head — a cable bill, a payroll line, an EHR subscription, the cost of one citation? What should we be comparing ourselves to?
- Does price signal quality to this buyer, or are they purely cost-driven? Is there a

number that's *too cheap* to be taken seriously in a compliance-sensitive market?

- (Only after she's given her own number) We've been modeling around \$149/month flat — does that land where you'd expect, too high, or leaving money on the table?

- On deal mechanics: we're giving the first 5 facilities free lifetime access for feedback. Smart, or does it anchor expectations and attract the wrong kind of pilot?

What's a better early-pilot offer?

6. Competition / channel risk

(Tests competitive-intelligence.md.)

- Synkwise already has ~1,500 facilities and is embedded in the associations.

Realistically — partner, or eventual competitor? If partner, how would you approach them?

- Caring Data was founded by an RCFE owner who holds an association advisory seat. How much does founder-credibility-in-the-channel matter versus the actual product here?

- Do operators tolerate running two systems (meds/compliance in one, documentation in another), or does someone inevitably consolidate it all?

7. Founder-led motion & scaling

(Tests the solo, founder-led plan to hit 50 facilities in ~6 months.)

- At what point does solo founder-led sales break in this market? What's the first sales/BD hire you'd make, and when?

- What would you personally want to see before you'd say this has real product-market fit in *this* segment specifically?

8. Closing — fit as an advisor / BD partner

(The "would I work with you / would you open doors" angle. Save for the back half.)

- If you were advising us, what's the first thing you'd want to own or change?
- What would make this worth your time — equity advisor, paid consulting, or intro-based / success-based?

- Are there 2–3 people in your network you'd want to introduce us to? What would you need from us to feel comfortable making those intros?

- What's the one assumption in our plan you'd bet money is wrong?